

A pulmonary research *institute.*

APRIM is a small, specialty-focused clinical research institute in Bloomfield Hills, Michigan. We run Phase 2–4 trials for pharmaceutical and biotech sponsors with industry-grade operational rigor and neighborhood-level patient care.

2019

FOUNDED

40+

TRIALS AS PI

98%

PHASE 3 RETENTION

§ MISSION

Research in *service* of breathing.

APRIM exists because too many of the people who live with COPD, asthma, or pulmonary fibrosis in southeast Michigan were being asked to drive an hour each way to participate in the research that might help them. We built an institute to meet them where they already are.

We focus exclusively on pulmonary medicine. Every study we take on is one our principal investigator has treated patients for in clinical practice. Every protocol is executed by coordinators trained in respiratory care, not generalists. Every participant gets the same face on every visit.

It is a narrow mandate. It is the only one we want.

§ 01 / TEAM

The *people* who answer the phone.

8 FTE • BLOOMFIELD HILLS, MI

DR. PARODI FOUNDER • PI Franco Parodi, MD Triple board-certified in Pulmonary, Critical Care, and Internal Medicine. 40+ trials as PI.	DR. HART SUB-INVESTIGATOR Meera Hart, DO Pulmonology. Sub-investigator on severe asthma and IPF programs. Fluent in Spanish.	R. WILKES RESEARCH DIRECTOR Rebecca Wilkes, RN, CCRC Oversees protocol execution, regulatory submissions, and participant safety.	T. LIN LEAD COORDINATOR Tomás Lin, CCRC Your first and most frequent point of contact. Runs study visits day-to-day.
J. OBI PFT SPECIALIST Joelle Obi, RPFT Certified pulmonary function technologist. 12 years in respiratory research.	A. REY PATIENT ADVOCATE Andrea Rey Helps participants navigate transportation, scheduling, and anything else we can fix.	M. SOTO REGULATORY Mateo Soto IRB submissions, sponsor reporting, and everything the FDA needs in a tidy binder.	S. BRAND OPERATIONS Simone Brand Keeps the clinic running, scheduling sharp, and our coffee machine in working order.

§ 02 / HISTORY

Seven years, *five moments.*

UPDATED • 2026

2019

APRIM founded in a single exam room. First COPD trial begins in November.

2020

Seven new trials activated. We learn to do home monitoring when visits become impossible.

2021

Dedicated 4,200 sq ft research suite opens in Bloomfield Hills. Dr. Hart joins as sub-I.

2023

500th participant enrolled. First IPF anti-fibrotic program completes Phase 2 readout.

2026

Three studies actively enrolling. Vaccine program expands. PMA partnership formalized.

§ 03 / CAPABILITIES

What we have *on-site.*

4,200 SQ FT • BLOOMFIELD HILLS

PULMONARY TESTING	Full PFT laboratory Spirometry • lung volumes • DLCO • 6-minute walk • FeNO	✓ Calibrated daily, NBRC-certified technologist
IMAGING	HRCT + chest radiography Via partner imaging center • 1.2 mi from site	✓ Same-day scheduling, secure sponsor transfer
SPECIMEN	IATA-certified specimen handling Central lab processing • on-site -80°C freezer • courier chain	✓ Sponsor-qualified for oncology-grade chain of custody
SYSTEMS	EDC • CTMS • eSource-ready Veeva • Medidata Rave • Oracle Siebel — compatible	✓ Part 11 compliant, typical activation 28 days
REGULATORY	Full IRB submission infrastructure Central + local IRB workflows • 1572/FDF on file	✓ Zero critical findings last 3 audits

♦ WORK WITH US

Let's start a *conversation.*

Whether you're a volunteer, a sponsor, or a referring physician, the same number reaches a real person at APRIM. No call tree, no triage queue.

See if you qualify

Talk to a coordinator

Where every *breath* becomes evidence.

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Active *trials.*

Every trial below is actively screening or enrolling. Tap any row for the full protocol summary, eligibility criteria, and a two-minute pre-screen to see if you qualify.

3

ENROLLING

3

IN CATALOG

40+

COMPLETED

[§ 01](#) / NOW ENROLLING

3 trials, *three therapeutic areas.*

[UPDATED](#) • [LIVE](#)

ID	STUDY	INDICATION	PHASE	AGES	STATUS	
APR-001	AURORA-1 — <i>Long-acting bronchodilator for moderate-to-severe COPD</i> A 24-week study of a once-daily inhaled therapy for adults with moderate-to-severe COPD. Compensation provided for time and travel.	COPD	Phase 3	40–80	ENROLLING	See →
APR-003	HELIX-3 — <i>Antifibrotic combination for idiopathic pulmonary fibrosis</i> A 26-week study of an investigational oral antifibrotic in adults with idiopathic pulmonary fibrosis (IPF) on stable background therapy.	IPF	Phase 2	50–85	ENROLLING	See →
APR-002	MERIDIAN-2 — <i>Biologic for severe eosinophilic asthma</i> A 52-week study of an investigational injectable biologic for adults with severe asthma uncontrolled on inhaled therapy.	Asthma	Phase 3	18–75	ENROLLING	See →

[§ 02](#) / WHAT TO EXPECT

The *first* conversation.

[10–15 MINUTES](#) • [PHONE](#)

If a study feels like a possible fit, the next step is a short phone call with one of our coordinators. You describe your breathing and current medications. We describe the study. Either side can decide it's not the right time, no pressure either way.

[See the full participant journey →](#)

If we both want to keep going, we'll schedule a screening visit at our Bloomfield Hills office — full pulmonary function testing, chart review with Dr. Parodi, and a plain-English walkthrough of the protocol. You only sign consent after you've had time to ask everything you want.

[♦ DON'T SEE ONE THAT FITS?](#)

We'll keep your info *on file.*

Studies open and close month to month. With your permission, we keep your details ready and reach out the moment something matches.

[Get on the list →](#)[About APRIM](#)

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COPD · RECRUITING

AURORA-1 — Long-acting bronchodilator for moderate-to-severe COPD

A 24-week study of a once-daily inhaled therapy for adults with moderate-to-severe COPD. Compensation provided for time and travel.

About the study

AURORA-1 evaluates an investigational once-daily inhaled therapy designed to improve lung function and reduce exacerbations in adults living with moderate-to-severe COPD. Participants attend study visits at our Bloomfield Hills office over approximately six months.

Who can participate

You may be eligible if you:

Are between 40 and 80 years old

Have a diagnosis of moderate-to-severe COPD

Have a smoking history of at least 10 pack-years

Are willing to attend seven on-site visits over 24 weeks

A research coordinator will confirm eligibility at a screening visit.

What to expect

An initial screening visit including spirometry and basic labs

Six follow-up visits with breathing tests and study medication review

Compensation for time and travel at each visit

ASTHMA · RECRUITING

MERIDIAN-2 — Biologic for severe eosinophilic asthma

A 52-week study of an investigational injectable biologic for adults with severe asthma uncontrolled on inhaled therapy.

About the study

MERIDIAN-2 is evaluating an investigational injectable biologic for adults with severe asthma whose symptoms persist despite high-dose inhaled corticosteroids. The study runs approximately one year with monthly visits.

Who can participate

You may be eligible if you:

Are 18–75 years old

Have a diagnosis of asthma

Are currently on a daily controller inhaler

Have had at least one asthma flare in the last year requiring oral steroids

What to expect

Monthly study visits over 52 weeks

Investigational study drug provided at no cost

Compensation for time and travel

IPF · RECRUITING

HELIX-3 — Antifibrotic combination for idiopathic pulmonary fibrosis

A 26-week study of an investigational oral antifibrotic in adults with idiopathic pulmonary fibrosis (IPF) on stable background therapy.

About the study

HELIX-3 evaluates an investigational oral antifibrotic, taken alongside standard therapy, in adults living with idiopathic pulmonary fibrosis. The study aims to measure changes in lung function (FVC) and patient-reported breathing symptoms over approximately six months.

Who can participate

You may be eligible if you:

Are 50–85 years old

Have a confirmed diagnosis of IPF

Are on stable pirfenidone or nintedanib for at least 12 weeks

Can attend nine on-site visits over 26 weeks

What to expect

Lung function testing (spirometry, DLCO) at each visit

Investigational drug provided at no cost

Compensation for time and travel

HOME / FOR PATIENTS

Participation, *demystified.*

Four steps, one clear timeline, no gotchas. Below is what actually happens from the first phone call to the day a study closes — plus the answers to the questions we get every week.

Free
TO SCREEN

\$40–
\$150
PER VISIT, TYPICAL

24h
COORDINATOR REPLY

§ 01 / STEPS

The *four* stages.

FROM INQUIRY TO CLOSEOUT

<i>i</i> DAY 1 • CALL A short, no-pressure conversation. 10–15 minutes with a coordinator. You describe your breathing, current meds, and history. We flag anything that might fit — and we say so plainly if nothing does.	<i>ii</i> WEEK 1–2 • SCREENING A 90-minute clinic visit. Full pulmonary function testing on-site, a chart review with Dr. Parodi, and a plain-English walkthrough of the protocol. You sign consent only when ready.	<i>iii</i> MONTHS 1–12 • STUDY Scheduled visits, covered costs. Typically monthly. Medication, labs, and imaging are covered by the sponsor. Compensation per protocol. You can withdraw at any point, for any reason.	<i>iv</i> MONTH 12+ • CLOSEOUT A warm handoff, not a drop-off. Final safety visit, written summary of what the study learned, and — where eligible — a path to open-label access to the therapy that was tested.
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§ 02 / SAFETY

Three *promises* on visit one.

IRB-MONITORED • 2024 AUDIT CLEAN

01 • INDEPENDENCE An independent review board watches over every study. No trial at APRIM runs without ongoing oversight from an IRB that has no financial stake in the outcome. If they pause a study, we pause it the same day.	02 • TRANSPARENCY Your data is yours, always. Lab results, imaging, pulmonary function scores — all of it. We send copies to your primary doctor and your pulmonologist on request. No black box.	03 • EXIT You can leave any study, any day. No questions, no guilt, no administrative delay. A short form, and your standard care continues uninterrupted. We stay friendly either way.
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§ 03 / FAQ

Questions we hear *every week.*

PLAIN ENGLISH • NO ACRONYMS

01	Is it <i>safe</i>?	Every study is reviewed by an independent IRB and continuously monitored. No first-in-human dosing happens at APRIM — only Phase 2+ programs with established safety data. You can withdraw at any time, for any reason.
02	What's the <i>cost</i>?	Zero. All study-related visits, medications, labs, and imaging are covered by the sponsor. Most studies also compensate participants for time and travel — typically \$40–\$150 per visit.
03	Do I keep my <i>own doctor</i>?	Yes. APRIM works alongside your primary care physician and pulmonologist. We send visit summaries to whoever you designate and coordinate directly on anything that changes.
04	What if I <i>don't qualify</i>?	We tell you clearly and quickly. With your permission, we keep your information on file for a future study that might be a better fit.
05	Will I get the <i>real drug</i>?	It depends on the study design. Some are placebo-controlled; others are open-label. We walk through exactly what each protocol involves during your screening visit — before you sign anything.
06	Can I <i>quit</i>?	Anytime. No questions, no penalty, no impact on your care. A short form, and your standard treatment resumes uninterrupted.

♦ READY WHEN YOU ARE

Start with a *conversation.*

Twenty minutes on the phone. No commitment, no consent forms, no paperwork. You leave knowing whether anything fits — and we leave knowing how to help.

[Call a coordinator →](#)

[See the studies](#)

Where every *breath* becomes evidence.

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HOME / FOR PHYSICIANS

Refer with *confidence.*

APRIM works alongside referring pulmonologists and primary care physicians, not around them. Every visit summary goes back to your office. Every protocol decision loops you in. Your patient stays your patient — we just open another door.

24h

REFERRAL ACKNOWLEDGED

Every visit

SUMMARY BACK TO YOU

0

COST TO YOUR PRACTICE

§ 01 / HOW REFERRALS WORK

Three steps, *nothing* more.

OR CALL (586) 980-1296

<i>i</i> YOU Send a name & number. Email the patient's name, phone, and the indication. That's the entire referral. We'll close the loop on the rest.	<i>ii</i> WE Reach out within 24 hours. A coordinator calls your patient, walks through what fits, schedules screening if it's the right move. You get an acknowledgement email same day.	<i>iii</i> TOGETHER Stay in the loop. Visit summaries, lab results, and protocol notes flow back to your EMR. If anything changes — meds, status, exit — you hear it from us first.	<i>iv</i> ALWAYS Their care, uninterrupted. We don't prescribe outside of protocol. We don't replace your care plan. The patient's relationship with you stays exactly where it was.
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♦ SEND A REFERRAL

One *email* is enough.

referrals@aprim.org — name, phone, indication. We take it from there.

Talk to a coordinator →

See current studies

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HOME / CONTACT

One *conversation* starts it all.

The phone number below goes to a real person on our coordinator team — no triage, no call tree. We answer weekdays 8 a.m. to 6 p.m. Eastern, and reply to email within one business day.

(586)
980-1296

Mon–Fri
8 A.M. – 6 P.M. ET

< 24h
EMAIL REPLY

S 01 / REACH US

Three direct *lines*.

BLOOMFIELD HILLS, MICHIGAN

<i>i</i> BY PHONE (586) 980-1296 A coordinator answers — same day, every day we're open. If you reach voicemail, we return calls the next business day.	<i>ii</i> BY EMAIL hello@aprim.org Reach the whole coordinator team at once. Best for non-urgent questions, study inquiries, or sponsor introductions.	<i>iii</i> IN PERSON Bloomfield Hills, MI Our research suite is open by appointment. Park free in the lot; the front desk hands you off to a coordinator on arrival.	<i>iv</i> FOR SPONSORS Site qualification Our regulatory packet, capability statement, and metrics dashboard are available on request. One email gets you all three.
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♦ OR BROWSE THE STUDIES

See what's *open* right now.

Each study page lists eligibility, visit cadence, and compensation.
A two-minute pre-screen tells you if you're a fit before any phone call.

See active studies →

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